

40. PRACTICE 1 THE COLIC FRAME

DURATION : 03:16

STUDY SHEET

COURSE SUMMARY

In this video, the practice of the colonic framework is explored through crucial support points such as the mesenteric artery and the left colic flexure. The speakers emphasise intestinal motility and its connection to the psychic and emotional planes, demonstrating how certain tensions can reflect internal imbalances such as ptosis or visceral dysfunctions. The importance of abdominal normotension is highlighted, addressing issues of hypertension and hypotension. The connections between the transverse colon and personal certainties, as well as the impact of dietary excesses on well-being, are also discussed, offering students and therapists keys to an integrative and liberating approach.

PRACTICE 1: THE COLIC FRAME

In this practice, we will explore the essential **fulcrums** in the colic frame.

The **first fulcrum** is the **mesenteric artery**. Then, the **second fulcrum** is located at the **left colic flexure**, which is fixed. The **third fulcrum** is located deep. Finally, the last fulcrum is linked to the dynamics of the intestine, which moves in the opposite direction.

The movement we observe is a **motility** movement. From this point, we can begin to trace a **story**. I am immediately drawn to the **stomach**, the **spleen**, and the left colic flexure. This fulcrum is rebalancing a rather deep **psychic plane**. It is possible that it is seeking to move closer to a more anterior **fulcrum** to retrieve information.

This point is in equilibrium between the **aortic plane** and the **splenic plane**. It is a plane of integration, an unconscious that desires to rise to the surface, potentially expressing itself as a **dream** or a **nightmare**. By placing the fulcrum on the left side, I am seeking to achieve a certain **freedom**.

The final point is located here. Normally, **epigastric tension** should be slightly greater than **hypogastric tension**. This is always a search for **abdominal normotension** or **abdominal hypotension**. If, during my palpation, I find that hypogastric tension is greater than epigastric tension, this indicates a system of **ptosis**.

In this case, I must work on a **viscerospatial** or fluid rebalancing system. In cases of **hypertension**, it is necessary to release the fluids. Conversely, for **hypotension**, a release is required, which implies reintegrating the viscerospatial system to restore **dynamics** and increase strength.

It is important to note that **dietary excesses** can still be improved. As I ascend, I observe an angle forming, and as I descend, I move towards my **zone of joy**. If problems are not resolved, this can lead to the **zone of anger**.

The **transverse colon** represents my **certainties** in life. If these certainties are not resolved, they turn into **doubts**. As I ascend, I face the **zone of trust**, where my **anxieties** reside. The angle here is what is called the **ball and chain of guilt**, which it is essential to clear.

Finally, as I descend, I address the **search for my independence**, a process by which I must be totally free from my **parents**.

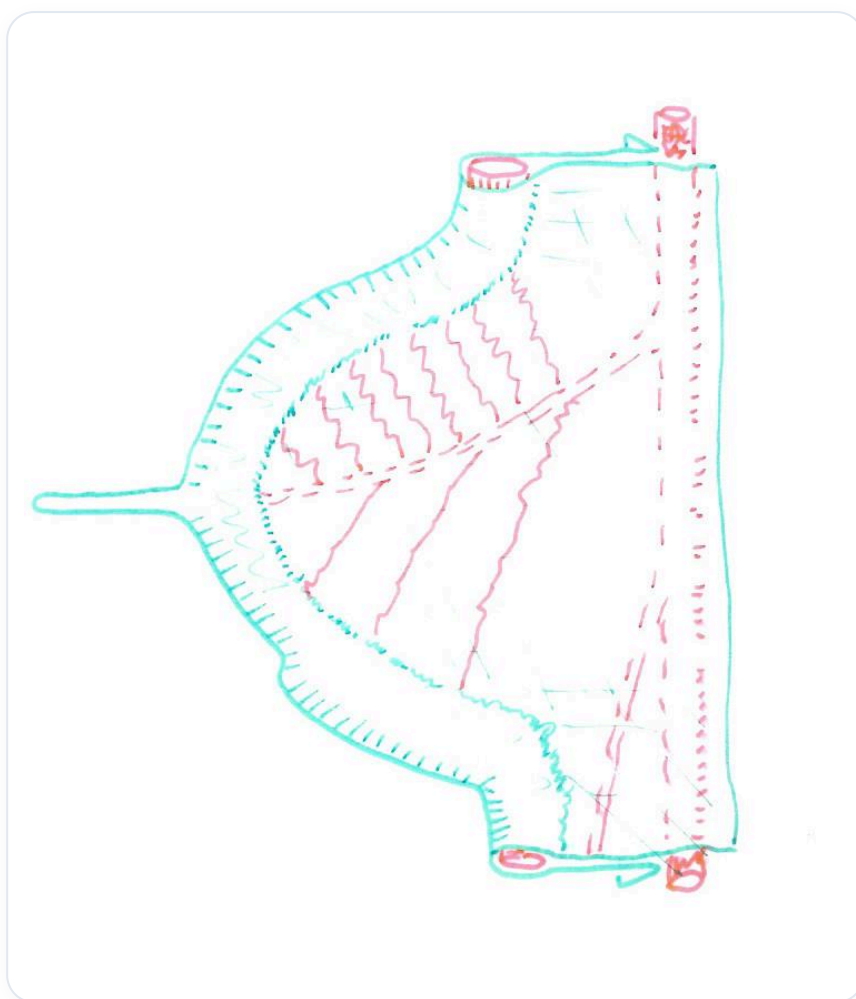


FIG. — Embryonic digestive tract